



Belletown Clinic Resident Handbook



Updated 6/2026. Click below to jump to sections or cmd/ctrl + f to search.

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The Basics

Welcome to your continuity clinic experience at UW Primary Care Belletown!

Clinic Address:

2505 2nd Ave., Suite 200, Seattle, WA 98121
(Located on 2nd Floor)

Helpful Phone Numbers:

UW Attendance Line	206-520-1529
Front Desk	206-443-0400 (number for patients) 206-520-1500 (direct front desk line – do not give to patients!)
Fax	206-520-1599

Important Contacts:

- UW Belltown Education Director
 - Samantha Alspach, MD (c: 206-412-5015, e: moensp@uw.edu)
- UW Belltown Clinic Manager
 - Carla Harding (c: 206-859-3429, e: carlahar@uw.edu)
- Chief Resident 2026-2027
 - Divya Takkellapati, MD (c: 973-220-9146, e: dtakkell@uw.edu)

Clinic Hours:

Clinic hours are Monday through Thursday 8am-6pm and Friday 8am-5pm. There is a small attending-led Saturday clinic from 8am-3pm. This can be used to schedule a patient if they need close follow-up, although urgent appointments are often limited.

Meet the Care Team

- **Patient Services Specialists (PSS) Team:** Operate front desk, help with medical records and paperwork
 - Lead: Shauna Fredrickson
 - Cindy Guillen, Erin Boudreau, Orion Rendon
- **Medical Assistants:** Help with rooming, in-visit tasks
 - Lead: Dulce Rivas-Jiminez
 - Cece Jasso, Christine Alvarez, Dax Bort, Haimanot Ayle, Justin Runquist, Ketema Nebra, Lucas (Takauto) Kimura, Maisha Bengi, Naema Ali, Nav (Navneet) Kaur, Sam Formo
- **Lab Team:**
 - Lead: Dan Lee
 - Chris Lee, G (Gagandeep) Dhaliwal
- **Between Visit Care coordinators (BVC):** First point of triage for phone calls, MyChart messages; can relay simple messages to patients for you.
 - Sesen Kidane, Quincie Haughton

- **Resident Flex ARNP:** Helps with inbox coverage, seeing patients when you are not available
 - Kasey Silver, ARNP
- **Nurse Care Manager:** helps with in-between visit diabetes care, injection training, complex care coordination
 - Elizabeth (Beth) Bierman
- **Behavioral Health Integrated Program Team (BHIP):** on-site, short to medium term therapy, psychiatry consults
 - BHIP Social Worker: Kimberly Butorac, MSW, LICSW
 - BHIP Psychiatrist: Jennifer Braden, MD, MPH
- **Nutrition:**
 - Deb Katz, MNS, RD (onsite on Wednesday, refer to Nutrition - UWPC Belltown)
- **Panel Navigator:**
 - Richi Thomas (rthomas8@uw.edu)

Clinic Spaces

- **Resident Pod 1 (Res A):** North Pod, Room 239
- **Resident Pod 2 (Res B, used when 2 attendings are present):** Room 222
- **Telemedicine Room:** Consult A
- **Lab:** Across from front desk in lobby
- **Supply Rooms:** Room 214, 236
- **BHIP Social Work (Kimberly's office):** Room 221B
- **BHIP Psychiatrist (Dr. Braden's office):** Consult C – only here on Mondays
- **Nutrition (Deb's office):** Consult C – only here on Wednesdays
- **Nurse Care Manager (Beth's office):** Consult E
- **Panel Navigator (Richi's office):** Consult B (near lab)
- **Admin Office (Carla, Sammi, Elizabeth, Chief Resident):** Staff Office, room 235
- **Men's Restroom:** Located past exam room 26 (room 240). (Staff code is different from patient code, which changes monthly.)
- **Women's Restroom:** From elevator turn left and go past room 222. (Staff code is different from patient code, which changes monthly.)
- **Additional Restroom:** Located in hallway between east pod and west pod
- **Kitchen, Coffee Machine:** Room 202
- **Conference Room:** Room 203

Transportation

- **Parking:** Residents often use public transit, biking, street parking, or public lots. There is no public parking available in the building. Nearby public parking lots generally have the best daily price. Apps that some residents use to reserve spots are called “SpotHero” and “ParkMobile”. Be careful with street parking payment windows, as they are generally strictly enforced.
 - **Bike Parking:** Bikes are not allowed in the clinic or lobby. If you bike to clinic, it is recommended that you park your bike in the locked garage for safety. To park in the garage, you need to purchase a fob for \$50. Complete the Garage Remote Form on the Belltown Teams and email it to Carla if interested.
 - **After Hours Access:** The clinic elevators open to the public at 7:45AM. Contact Carla to acquire the fob that allows access during non-business hours. The building front door is unlocked outside of business hours. If you are the first to arrive in clinic, an alarm will sound. **If you anticipate arriving early, please ask Carla for a code for the alarm.**
-

Clinic Workflow

Schedule:

From June to December, R1s will have 60-minute visits with 3 patients per half-day. Following this, all residents have 40-minute visits with 5 patients per half-day. This includes time for staffing.

Arrive at least 20 minutes prior to first appointment start time for MA huddle. You are expected to be in clinic on time even if your patients are not initially on your schedule, as open slots often fill up last minute. Be present for afternoon teaching sessions at 1pm.

Room Locations:

Workrooms are assigned based on the number of residents present in clinic on any given day. On days when 2 attendings are precepting (certain Mondays/Fridays), residents will be assigned to either the main resident workroom, Room 239/Res A or in Room 222/Res B. You will receive an email from Carla called the Belltown Daily that lists your room assignment and the attending you are working with on the day before clinic. If you don't receive the email, check in with the front desk when you arrive.

Staffing:

Patients are informed that this is a teaching clinic and that you will work with a supervising physician. It is still important that you explain the workflow to them. Please let your patient know that it can take up to 20 minutes to staff. If it is taking longer, please return to the room to update the patient. The MAs do not know how long staffing will take you, so they cannot update the patient with an estimated time.

While staffing, always keep the workroom door closed to protect patient confidentiality. Avoid loud conversations while staffing in the main resident workroom, Room 239/Res A, as patients can hear loud conversations in the adjacent rooms.

All patients must be staffed with the attending before the patient leaves the clinic or the telemedicine visit is ended. This is necessary for patient safety and for billing purposes.

Attendings must see the patient in the following situations:

- All visits with R1s from June through December
- E4/N4 or higher billing level visits
- TCM visits (post-hospital discharge visits)
- Medicare annual wellness visits (generally advised but not required)
- Attendings must be present during all procedures, including pap smears, in order to bill for them

Clinic Observations:

R1s will be observed on key outpatient skills throughout the first 6 months. These include agenda setting, history, physical exam, counseling on new medication, counseling on behavior change, counseling on screening/prevention. Each time you complete a clinic observation, please do the following:

1. Place a sticker on the chart on the wall
2. Log a Procedure/Case in MedHub. Go to MedHub > Procedures. List clinic location, supervising attending, and Add Standard Procedure > Continuity Clinic.

You **must complete all clinic observations** to be cleared to see patients independently after 6 months.

Notes:

Complete all notes and sign visits within 72 hours. Ideally, your notes should be completed on the same day as your visit.

Dress Code:

Business casual, closed-toed shoes. Please dress professionally while in the clinic, even if you are not seeing patients that day.

Telemedicine Visits:

You can complete telemedicine visits in Consult A. If already in use, talk to your attending or MAs for an alternative location.

If your patient has not joined zoom for the visit at the time of the appointment, please call the patient (using one of the anonymous call options) to ask them to log in. See Clinic-Related Tools > Calling Patients.

If you are running behind and the patient is signed in, please notify them using the Epic “Send Provider Late” tool. This is faster and easier than asking the front desk team to do so.

- Schedule > Events > Send Provider Late

Intervisit Patient Care Curriculum:

The Intervisit Patient Care Curriculum consists of continuity clinic days dedicated to Population Health topics, Inbox Management, or alternative learning experiences. You will receive more information on the curriculum. Inbox Management involves taking care of items in your Epic inbox. It is expected that you are onsite at clinic for Population Health and Inbox Management days. You can ask the precepting attending for help with inbox items as needed.

Flags, Cards, and Dots

Flags and cards outside each exam room and dots in Epic are used to communicate about the status of the visit.

Flags

Raised flags indicate the following:

(The exact order of flags on the wall varies.)

Black	Patient in room
Yellow	MA needed (if you request EKG, vaccines, BP recheck etc.)
Green	Not really used
Orange	Specimen in room for MA's to manage
Blue	Next patient (if >1 patient roomed)
Red	Provider in room (open flag when you enter and close it after)



If MA assistance is needed for vaccines, EKG, ear lavage, orthostatic vital signs, etc.:

- Sign the order (cannot be completed until signed). You can use “Selective Sign” and return to sign the other orders later.
- Open the yellow flag
- Notify an MA in your pod

Ear lavage orders:

- With Debrox: Clinic-Administered Medications: carbamide peroxide (Debrox) 6.5% otic solution + Ear Cerumen Removal (aka LAVAGE)
- With water only: Ear Cerumen Removal (aka LAVAGE)

Cards

The cards are located in the bin outside exam rooms.

Red card - signifies that the patient is changing or a sensitive exam is occurring. Avoid entering when the red card is on the door. If you need to enter, knock loudly and confirm the patient is ready before you enter. If you are performing a sensitive exam, place the red card on the door.

Purple card - if a patient needs a Pap test, but you don't have time at the visit, one option is to give them a purple card. They can take this to the front desk, who will know that they need to be scheduled for a dedicated Pap visit with an NP. You can also ask them to schedule with you for a dedicated visit. See Procedures > Cervical Cancer Screening.

Dots

Dots on your Epic schedule indicate the following:

	MA's have completed forecasting
	Rooming in progress
	Ready to be seen by provider
	Orders signed for action items for MA
	Order being completed by an MA

When status is listed as "Ready", this means the patient is ready to be roomed, not to be seen by the provider. The green dot is the only marker that indicates the patient is ready to be seen by the provider.

If you are active in a chart after a patient has checked in and before they have a white dot, the status may change to "Visit Complete." After you leave the chart, please check the status. You can right click it and change it back to "ready" or "waiting." If the visit says "Visit Complete" and there is no white dot, the MAs may think that the patient does not need to be roomed.

Inbox Management

Daily Inbox Management

Urgent Coverage: On ambulatory rotations, consults, and elective rotations (excluding WWAMI), you are expected to manage your inbox daily. An attending and/or flex APP looks at your inbox on business days to address urgent matters only. This is referred to as “Urgent Coverage”. They will leave things that can wait more than 24 hours in the inbox.

- Please check your inbox every weekday and address any unaddressed patient care items
- All results should be addressed with a “Result Note” for clear tracking of what has been addressed for covering providers
- If you have questions about a result or plan, please see the Belltown Weekly email to find out who is attending that day
 - o You can send an inbox staff message (Epic chat message if urgent) directly to that provider
 - o Or you can call the Belltown Attending Desk Line (206-520-1593)
 - o Or you can send a staff message (Epic chat message if urgent) to the provider with whom you staffed the patient

Prior to Absence from Clinic

Full Coverage: On inpatient admitting rotations, ED rotations, WWAMI rotations, Risk, vacation, and personal leave, you are not expected to manage your inbox daily. Your inbox will be covered by attendings or other clinic providers. This is referred to as “Full Coverage”. Inpatient consult rotations, such as H Geriatrics, and electives do not receive full coverage. For VA Cardiology, the two weeks of consults have full coverage, while the 2 weeks in clinic do not. An attending will be looking at your inbox during business days and will address all time sensitive matters. They will leave items that can wait until you return.

- Please clear your inbox completely prior to absence
- For results that you have not yet addressed, please leave a “Result Note” briefly describing your plan
- For anything you want to remain in your inbox, please forward to yourself as a staff message
- If you have a day in clinic during an admitting or ED rotation, you are expected to **manage all results and follow-up items related to the clinic day during the 3 business days following the clinic day**
- If an inbox situation arises in which it would be ideal for the PCP to contact the patient (for example, a new cancer diagnosis), the covering provider or the Chief Resident will contact you to see if you have availability to contact the patient

In general, if you do not address items from a clinic visit within 3 business days, or if there are other inbox items that have not been addressed in a timely way, the Chief Resident or an attending may email you to remind you to complete them.

Clinic Days

You are expected to cover your own inbox when you are in clinic. **Attendings/ARNPs are not attached to your inbox when you are in clinic. You are expected to fully clear your Epic inbox and your physical inbox (faxes, records, paperwork, etc.) prior to leaving the clinic.** If you have a clinic day but you leave an item in your inbox that only you can address, we may ask you to return to clinic to complete this item, even if you are on an inpatient rotation.

Full Coverage Rotations:

ER, H CCU, H MICU, H Neuro, H Wards, Night Med, Swing, U Cards, U Heme/Onc, U MICU, U Wards, VA Cards (consult weeks), VA MICU, VA Wards

Urgent Coverage Rotations:

All electives, BELL outpatient, H CCU (consult weeks), H Geri, Immersion, VA Cards (clinic weeks)

Pre-Immersion Coverage:

R1s have Full Coverage until the beginning of their Immersion block.

Prior Authorizations:

Prior authorizations are completed by teams inside and outside the clinic working with the insurance company. It is important to let patients know that it typically takes **at least several weeks** for a prior authorization to be completed and that the prior authorization may be denied. Completion time depends on prior authorization volume.

GLP1 medications have specific indications and often require prior authorizations with specific documentation. The dotphrase .GLP1 can be helpful for checking indications and documenting the necessary information.

Sending messages to pools:

When responding to messages from clinic staff, please respond to the entire pool. This ensures that a member of the team will be able to see the message, regardless of whether the sender is in clinic. Please also respond to the entire pool for messages from other departments, such as referral pools.

UW Belltown Clinic pools:

P UWPC BELLTOWN FRONT DESK POOL	→ Front desk team
P UWPC BELLTOWN CLINICAL SUPPORT POOL	→ Between visit care team
P UWPC BELLTOWN MA POOL	→ MA team
P UWPC BELLTOWN LAB/RAD POOL	→ Lab team
P UWPC BELLTOWN TEAM POOL	→ Combination pool for paperwork

- Please send all messages regarding paperwork to the P UWPC BELLTOWN TEAM POOL, which is a combination of Front desk team, Between visit care team, and MA team. This helps us keep track of paperwork volume in the clinic and allows any available team member to address it.

How to send a message to a pool:

Under To: enter pool name

You can search for our pools by entering Belltown:

The screenshot shows the 'To:' field with the text 'belltown' entered. Below the field, a dropdown menu titled 'Recent Recipients' is displayed. It contains a table with the following data:

Name
UWPC BELLTOWN FRONT DESK POOL [10036]
UWPC BELLTOWN CLINICAL SUPPORT POOL [17001]
UWPC BELLTOWN LAB/RAD POOL [10064]
UWPC BELLTOWN MA POOL [100133]

You can also search by clicking on the magnifying glass:

The screenshot shows the 'Enter recipients' field. A magnifying glass icon is highlighted with a red box. Below the field, there is a 'Pool for Responses' section with an 'Add a pool' button and a 'More' dropdown menu.

In the pop-up, search in the Pools field. Be sure to click **Add To**, which will add the pool to the **To Recipients** in the bottom left. Then click **Accept**.

The screenshot shows the 'In Basket Recipients' pop-up window. It has a 'Pools' field with 'cardiology' entered. Below it, a 'Search Results' table is displayed with the following data:

%	Registry Name	Registry ID
☐	UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY E PCC POOL	508100104
☒	UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY ECHO FRONT DESK P...	500100001
☐	UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY ECHO PCC POOL	500100002
☐	UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY ELECTROPHYSIOLOG...	5112190261
☐	UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY EP CLINICAL POOL	60000119
☐	UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY GEN CARD CLINICAL ...	60000118

Below the table, there are buttons for '+ Add To', '+ Add 'Cc'', and '- Remove 'Cc''. The '+ Add To' button is highlighted with a red box. Below the table, there is a 'To Recipients' section with a list of recipients. The first recipient is 'UWMC MONTLAKE HEART IN... 500100001'. Below it, there is a '- Remove 'To'' button. At the bottom right, there are buttons for 'Restore', 'Accept', and 'Cancel'. The 'Accept' button is highlighted with a red box.

The pool will now be present in the recipients.

The screenshot shows the 'To:' field with the text 'UWMC MONTLAKE HEART INSTITUTE CARDIOLOGY ECHO FRONT DESK POOL' entered. The text is highlighted with a blue box.

Physical Inbox

Your physical inbox is located in the mail room adjacent to the front desk. Paperwork that requires your attention will be placed in a purple folder in your inbox. You are expected to clear out the inbox on your days in clinic. If there is a time-sensitive matter in your inbox and you are not scheduled to be in clinic, contact your paired mentor attending to come up with a plan for completion of the item.

After completing the paperwork, place the items in the **Provider Outbox**. The outbox is located in the **center of the mail/copy room**. A 2nd provider outbox is located **outside the main resident pod, Res A, room 239**. Empty purple folders are found in a file bin at the entrance to the front desk. **Do not place purple folders containing papers in the front desk file bin.**

Absences and Schedule Changes

Unexpected Absences:

If you cannot make it to clinic because you are sick or have another reason for an unexpected absence, complete the following:

1. Call the UW Attendance Line at **206-520-1529** and leave a voicemail. This voicemail is forwarded to Carla. **Give as much notice as possible** to facilitate patient rescheduling.
2. Complete an **unexpected absence form** on the Internal Medicine Residency intranet page. Search intranet for “unexpected absence” to find the form.

Schedule Changes:

All planned schedule changes must be requested at least **60 days beforehand**. Email the clinic Medical Education Director (Samantha Alspach), Chief Resident, and any other impacted rotation directors to request the schedule change. If the changes are approved, complete the **schedule change request form** on the Internal Medicine residency intranet page.

The ability to make schedule changes depends on your number of clinic days scheduled, how many patients are already scheduled, attending schedules, clinic space availability, and additional factors. Not all requests can be accommodated. Giving as much notice as possible can help to increase the likelihood of approval. Dr. Alspach processes schedule change requests on Wednesdays and does not process them on the weekend.

Every resident has a minimum number of required clinic days in residency. See the Clinic-Related Tools section for the tool to check your number of scheduled clinic days.

Truncated Clinic:

On admitting rotations, your clinic days will be truncated. This means your first and last appointment slots will be blocked to give time for panel management and transition back to

inpatient. Clinic is not truncated on half-days. You may consider checking your schedule in advance (click “Open Slots”) to confirm these spots are blocked.

Performing Procedures

R1s must be supervised by the attending for all procedures. For all residents, Pap tests/cervical cancer screening and cryotherapy must be supervised to be billed, and attendings will supervise these procedures for R2s and R3s whenever possible. Please log all procedures in MedHub.

Cervical cancer screening:

You should aim to complete Pap tests for your patients yourself. As above, R1s must be supervised by the attending for all Pap tests, and Pap tests must be supervised to be billed.

If you do not have time to complete the Pap test in the current visit, you have 2 options:

1. Have the patient schedule a dedicated visit with you to complete a Pap test.
2. Give the patient the **Purple Card**, which is in the bin outside the clinic room door, to take to the front desk. The Front desk team will know to schedule the patient with Kasey Silver or another ARNP for a dedicated Pap test visit.

Self Pap test:

The standard protocol for cervical cancer screening at UW is a standard clinician-performed speculum exam with sample collection. If there is a strong reason why a Pap test should not be performed, the patient may be a candidate for an HPV-Self Swab test, which needs to be completed in the clinic. If this is the case, discuss the situation with your attending. Always check with your attending before completing cervical cancer screening.

MA Tasks

Forecasting and Huddle:

Before the visit, MAs will write information about healthcare maintenance items that appear to be due in the “Notes” section of the Schedule. This is called forecasting. When the patients checks in, the Front Desk will add information about what the patient is wearing to the Notes, so that MAs are able to identify the patient for rooming.

15 minutes before your first patient, an MA will come to the pod to discuss any relevant information about patients with you. This is referred to as “MA huddle”. This may include additional instructions for the MA, such as giving the patient a PHQ9 and GAD7 form during rooming. You can also add information to the Notes section of your schedule or discuss with the MAs later.

Rooming:

During rooming, the MAs will go over allergies, medications, and smoking history. For new patients, they will also review alcohol and substance use and sexual history. They may pend medication orders that the patient requests a refill for.

Gowning:

By default, patients are asked by MAs to change into a gown for the following reasons:

1. Gynecological concerns
2. Women's wellness visit with Pap test

Patients are not asked to change into a gown for a women's wellness visit without a Pap test or men's wellness visit.

If you would like a different approach than the standard protocol, please let the MA know during the huddle or indicate "Gown" or "No gown" in the Notes section of the schedule.

MA Procedure Orders

An order is needed before MAs can complete vaccines, EKG, ear lavage, or orthostatic vital signs. Please sign the order, open the Yellow Flag, and let an MA know.

Lab Tips

Completing Labs in Clinic:

Send patients to the front of the lab to check in. Sign all lab orders before patients go to the lab. If patients need to return to Belltown Clinic for a lab test, they should schedule a lab visit. They can do this at our front desk, by calling the clinic phone number, or you can send them a MyChart message with a button to schedule a lab visit. Use the Belltown lab/Xray dotphrase to give patients information on completing future labs.

Lab Order Information:

To find out more information about a lab order, you can go to the UW Medicine Lab Test Guide. Search for the lab, click Accept, and then click the order to open the pop up. Click the link to the UW Medicine Lab Test Guide. This is also on GHitOOLS > Additional Toolkits > UW Lab Test Guide

Lab Add-Ons:

To place a lab add-on order, communicate with the lab team. They will instruct you to add a regular order if the sample is still at Belltown or a lab add-on order if the sample has already been sent to Montlake or HMC. Lab changes before 2pm usually do not require a lab add-on order.

Canceling Labs:

If you cancel a lab, inform the lab team either by sending a message to the lab pool or by going by in person and letting them know. Lab pool: UWPC BELLTOWN LAB/RAD POOL

Time-Sensitive Labs:

When placing orders for time-sensitive labs, such as hormone levels, include the required collection time in the order comments. If it is already past the appropriate collection window (for example, ordering a testosterone level at 11 AM when it should be drawn between 8-10 AM), set the lab date to “tomorrow” or a subsequent day. Please also communicate with the lab team about which labs should be collected during the visit and which are for future collection. If you do not communicate with the lab team, they often will draw all the labs ordered during the visit.

Lab Test Swabs:

Be sure to always use the correct swab for your lab test. Most exam rooms have a Swab Guide Sheet inside the door of the cabinet above the sink. You can always check with your attending or the lab team if you are unsure about which swab to use.

Here are the lab codes for some commonly ordered swab tests:

- Sending sample for KOH prep: KOH (Direct Exam Fungal)
- Skin HSV test: RPDHSV
- COVID and influenza PCR: NCVFLU
- POC rapid influenza: POC109
- Influenza and RSV PCR: FLURSV

H. pylori Breath Test:

Patients undergoing H. pylori testing must be fasting, including water, for 1 hour and must not have taken a PPI, Pepto-Bismol, or antibiotics for 2 weeks prior to the test. It is ok if they have taken H2 blockers (famotidine).

Urinalysis:

The correct order for a urinalysis in clinic is U/A Auto Dipstick Only w/ Reflex Culture, Onsite

Infusion Orders:

Infusion orders, such as iron infusions or zoledronic acid, can be placed by searching for “Therapy Plans - Outpatient”. Click + Therapy Plan 1. Then search for the specific infusion. Check with your attending for help.

Support Services

- **Radiology:** There is no radiology on-site. For X-rays, the front desk can directly schedule patients into the radiology schedule at the SLU clinic which is a 5-minute drive/15-minute walk away. Patients can also walk in for X-rays or schedule more complex scans at any of our hospital locations including UW Roosevelt and UW Eastside Specialty Center (ESC). Use the Belltown lab/X-ray dotphrase to provide patients with information on completing Xrays.
- **Pharmacy:** No pharmacy is on-site. You can contact UWMC Montlake pharmacy for medication questions. There is a pharmacy at UW SLU if patients would like a nearby UW pharmacy. If you send medications to a UW pharmacy, the pharmacists can Epic Chat you with questions about the medication. UW Montlake Outpatient Pharmacy is a compounding pharmacy, which is helpful for compounded medications.
- **Social Work (non-BHIP):** Offsite, refer to Social Work for non-BHIP/mental health related social work needs.
- **Anticoagulation Clinic:** Pharmacist team at Montlake, patients must have established care with UW Cardiology or UW Hematology before they can be referred there. If a patient is not established with these specialties, we manage their INR at Belltown.

Behavioral Health Integrated Program (BHIP)

BHIP is a short-term therapy service embedded across all UW Primary Care clinics. The integration aspect refers to the model of care: Kimberly, BHIP social worker, is physically located on-site at the Belltown Clinic and works closely with you all during a patient's episode of care. This collaborative model helps with access to care, patient-centered care, reducing stigma, and lowering cost in some situations. It has been shown to have a shorter median time to remission of depression.

Patients can be referred to BHIP for the following reasons:

1. Short-term CBT (3-6 months)
2. One time psychiatry consultation for medication management with Dr. Braden
3. Help accessing community mental health resources

When offering BHIP referral to a patient it is important to cover these topics:

1. BHIP provides short-term care
 2. Patient needs to consent to referral, BHIP often has a copay
 3. For CBT, patient needs to be able to commit to several sessions
-

Psychiatry Consultation Line

If discussing the case with a psychiatrist would be helpful during a patient visit, one option is to contact the Psychiatry Consultation Line. You will be connected with a UW psychiatrist, typically within a few minutes. This service is similar to consulting other services inpatient though the psychiatrist does not review the chart and will base their recommendation solely on your conversation. You will later receive an email from the psychiatrist summarizing your discussion. This service can be very helpful for on-the-spot questions, such as for psychiatric medication side effects or cross-titration planning. Be sure to budget your time appropriately if calling during a clinic day. You can also call outside of a visit and discuss a patient scenario.

Psychiatry Consultation Line: **877-927-7924**

Nutrition Referrals

Deb Katz is the in-house Belltown dietician, and you can refer patients to her for nutrition-related needs. The following are patient situations in which a referral to Nutrition can be helpful:

- Issues eating a healthy diet
- Having trouble gaining weight
- Anxiety around foods
- Non-specific IBS type symptoms
- Starting GLP-1s
- Discuss supplements

A Nutrition referral is *not* helpful for counseling on eating disorders or for long-term meal plans. Patients with eating disorders need to see a comprehensive team at an eating disorder clinic outside of UW.

Medicare will only cover referral to Nutrition for diabetes (not prediabetes), CKD, and BMI ≥ 30 . Medicaid coverage for referral to Nutrition is broader, and includes diabetes, BMI ≥ 30 , and cardiovascular risk factors.

Nurse Care Manager Visits

You can schedule patients with the in-house Nurse Care Manager, Beth Bierman for check-ins between visits. For example, a patient with diabetes could meet with Beth between your visits to check in on blood sugar and insulin use and make small adjustments if needed. Beth can also do injection teaching, home blood pressure machine calibration, and complex care coordination.

To refer a patient to be seen, place a “Care Management” referral and choose “UWPC Belltown Family Medicine”. In the Comments, write that you are referring to Beth and specify the reason. Send a staff message to Beth if you’d like to discuss in more depth. Check with your attending when considering a referral.

Transgender Care Consult

Dr. Corinne Heinen, a Family Medicine physician who attends in Belltown Clinic, is an expert in transgender care. If you have more advanced questions related to transgender care for your patient, you can refer your patient for a one-time consultation with Dr. Heinen.

Place a referral to “Family Medicine” and choose “UWPC Belltown Family Medicine”. Specify in the Comments that this is for a one-time consultation with Dr. Heinen. See GHitOOLS for links on Transgender Care.

Late Patient Policy

Established patients:

When established patients arrive late but within their appointment timeframe, the front desk team will let the patient know that the visit may be shorter, and the patient will be roomed by an MA as soon as possible.

For established patients who arrive after their appointment timeframe, the front desk team will check in with you about whether it is feasible for you to see the patient. Talk with your attending if this comes up. It generally is reasonable to reschedule.

New patients:

For new patients who arrive late but before the halfway point of the appointment timeframe, we do our best to try to see them. New patients who arrive after the halfway point of the appointment timeframe will be asked to reschedule.

Booking Patients, Calling Patients

Booking patients:

You can book your patients for future visits using the “Book It” tab in Epic. You will see that your clinic days have hold slots in the schedule. These are used to prevent patients who are not on your panel from filling up spots.

If you complete a short **scheduling module**, you can override these holds. Request the module from Carla. Otherwise, the front desk can override these holds.

In general patients can schedule visits in these ways:

- At the front desk
- Calling the front desk
- Sending a MyChart message and receiving a scheduling ticket
- Signing up on their own on MyChart

Note that with **the first 3 options**, patients are able to **override holds**. If they sign up on their own on MyChart, they cannot. If your patient is having difficulty scheduling with you, you can request that they use one of the first 3 options for more availability.

Calling patients:

The BVC team or the Front Desk team can call patients on your behalf. They can communicate simple messages though cannot communicate complex medical interpretations and cannot answer medical questions.

To request that the Front Desk team call your patient to schedule a visit, create a telephone encounter and in a note enter the dotphrase .schedulingfd. This will pull up a template where you can list if the visit should be PCP only, type of visit, and timeframe for visit. You may also list the reason for visit.

There are 2 options for calling patients yourself anonymously:

1. Doximity app - free app, requires account creation. Allows you to use different caller numbers depending on your location, such as Belltown Clinic, UWMC Montlake, etc.
 2. [UW Medicine Caller ID Feature Request](#)
-

Belltown Code Protocol

A Belltown Code occurs when a patient becomes acutely ill. This rarely occurs and most often is due to vasovagal syncope (usually after a blood draw).

Belltown Code Instructions:

- 1 provider leads the code (MD, PA, or NP). Typically, this is the provider seeing the patient that day or the first provider to arrive.
 - If you are leading the code, find an attending to supervise
 - Have your badge visible and introduce yourself as a resident provider
 - If the code occurs in the lobby, move the patient into an exam room when safe
-

Clinic-Related Tools

Haiku, Canto, Dragon, and Imprivata Setup:

Haiku and Canto are Epic apps for phones and tablets. Instructions for setting these up are in the documents on the Belltown Teams.

Dragon is a dictation tool, which we recommend you try early on. Instructions for setup are in the documents on the Belltown Teams.

Imprivata is an app that is necessary for prescription of controlled substances. To set this up, download the Imprivata app and then check in with Carla before your first day of clinic.

More information on Haiku, Canto, and Dragon:

<https://sites.uw.edu/uwgme/ehr-training/uw-medicine-epic-ehr/>

Changing Your Name in Outlook:

1. Go to: <https://identity.uw.edu/>
2. Update "Preferred Name" to what you'd like displayed. Consider adding "Internal Medicine Resident" or your degree after your name to differentiate yourself from other accounts with similar names.
3. Now you should be searchable by full name. It could take up to a week to update.

GHitOOLS website: joshbloomstein.github.io/ghitools

Links to medicine resources and tools.

Galen's Continuity Clinic Calculator: <https://gaog94.github.io/GHITKU-Leave-Days/>

Calculates number of clinic days and leave days on your schedule.

Open Elective Searcher: <https://joshbloomstein.github.io/amion/>

This app searches for potentially available electives you could switch to by block.

If you are assigned an elective rotation that doesn't interest you, it may be possible in certain circumstances to switch electives. You could potentially do a straight swap with another resident or you could find an open elective. The ability to switch electives depends on multiple factors and cannot always be accommodated. I recommend giving as much advance notice as possible. Email the residency scheduling team at schedim@uw.edu if you have identified a potential switch.

Epic Optimization: [Epic and CPRS Efficiency AHD](#)

A collection of tutorials on Epic.

- Overview pdf: AHD Epic Optimization 2025 Josh Bloomstein.pdf.

Article Summary mp3s: [Article summaries](#)

Brief summaries of recent articles from top journals. Under further development.

Internal Medicine Podcast List

Rainier Tier

[NEJM This Week](#)

[NEJM Interviews](#)

[JAMA Medical News](#)

[JAMA Editors' Summary](#)

[JAMAevidence JAMA Guide to Statistics and Methods](#)

[The Clinical Problem Solvers](#)

[UW School of Medicine Faculty Thrivecast](#)

[Mayo Clinic Talks](#)

Serene Lake Tier

[The Curbsiders](#)

[Core IM](#)

[BMJ Best Practice](#)

[Harrison's PodClass](#)

Rattlesnake Tier

[The Lancet Voice](#)

[Annals of Internal Medicine Podcast](#)



High Yield Dotphrases

Remember to confirm that dotphrases are accurate and up to date.

System Dotphrases:

These dotphrases are automatically accessible for everyone.

.? - Searches your dotphrases or system dotphrases. Or search by clicking this button at the top of note:



.BMIE - Most recent BMI

.DIAGDAN - Pulls in all visit diagnoses and orders

- Has space to type the assessment
- Orders are refreshable
- You can move the diagnoses up and down or lump them
- If the encounter diagnosis is in the problem list, the A&P gets saved to the problem
- Previous A&P for that problem can be pulled into the note

.DICTATELATER - Allows you to dictate into notes through Haiku. Ideal for inpatient prerounding

.FIB4SCORE - calculate FIB4 + interpretation for the score

.FOLLOWUP - Pulls in text from the Plan > Follow-up tab. I have this in my note and AVS templates and write it in patient-centered language.

.GAD7SCOREONLY - Last GAD7

.PHQ9SCORE - Last PHQ9, refreshable

.PHQ9GAD7 - Last PHQ9 and GAD7, refreshable

.GLP - Indications for GLPs, though not updated for hepatic steatosis

.LASTEKG - Last EKG

.LASTECH - Last echo

.PQUES - Loads the patient questionnaire from current visit

.RISRSLTIMP1DREF - Gives all imaging impressions from the last 24 hours and is refreshable. Clean and concise, useful on inpatient.

.SOCDOC - Pulls in comments on Social History. To update this go to History → Social Documentation.

User Dotphrases:

In SmartPhrase Manager, search for the user to add their dotphrases:

Samantha Alspach:

.BELLLABXRAY - Information for patients on completing labs and x-rays

.BELLSPSYCHTHERAPY - Local community psychiatry resources

Note templates:

.BELLNOTEAWV
.BELLNOTENEW
.BELLNOTEReturn
.BELLNOTETELE
.BELLWELLMEN
.BELLWELLTGNB
.BELLWELLWOMEN

.SMAAVSBPLIFE - Lifestyle management guidelines for lowering blood pressure

AVS dotphrases: start with .SMAAVS

.SMADERM - Local community dermatologists

Jennifer Azen:

Alcohol use dotphrases:

.AUDCRITERIA
.HARRTSAFERUSEALCOHOL

Adrienne Clark:

.ACESA - Emotional support animal statement
.ACINSTINSOMNIA - Online tools for insomnia
.ACRESUOSTEO - Osteoporosis AVS

Lianne Hirano:

.LHDELIRIUM - Inpatient delirium treatment

Joshua Bloomstein:

.JDB4OPHPI (synonym .JDBOPHPI) - Smartlist of HPI dotphrases for common outpatient chief concerns in vanishing tip format

To use the smartlist:

Add all dotphrases that start with .JDB4

(click first one and hold shift while clicking last one to select and add all at once)

Then type .JDB4OPHPI.

Similar A&P smartlists for:

Wards - .JDB0WARDSAP

MICU - .JDB1MICU

Cardiology - .JDB2CARDS
Hematology/Oncology - .JDB3ONC

.JDB4FATIGUE - (synonym .JDBFATIGUE) - brief HPI, diagnostic framework, initial workup, ME/CFS diagnostic criteria

.JDBAOM - Brief acute otitis media HPI vanishing tip

.JDBAVS - AVS template

.JDBAVSCONTACTDERM - Contact dermatitis AVS, adapted from recs from dermatologist Dr. Lindsay Gunnell

.JDBAVSGASFOODS - Brief AVS for foods that can cause bloating

.JDBAVSGERDTRIGGERS - Brief AVS for GERD triggers

.JDBAVSHICCUP - AVS for preventing and stopping hiccups

.JDBAVSIRONSUPPLEMENT - Iron supplementation AVS, contributions from Dr. Astrid Strasik

.JDBAVSMAGFOOD - AVS for high magnesium foods

.JDBAVSSTATIS - Brief patient instructions for stasis dermatitis

.JDBBELLTOWN - Belltown Clinic emblem you can use in your note



.JDBBPDIET - OmniHeart diet for lowering blood pressure, dotphrase from Dr. Davonna Cufley

.JDBCATATONIA - Brief tips on diagnosis and treatment

.JDBCCLINICTIPS - Some bullet points of clinic and Epic information

.JDBDRYSKIN - Dry skin care

.JDBGLP - GLP HPI dotphrase with sections needed for our prior authorization process

.JDBHOMEBP - Home BP measurement instructions

.JDBHTNLOWER - Lifestyle measures for lowering blood pressure, dotphrase from Dr. Mack Holmberg

.JDBIOP to - Dotphrase for psychiatry intensive outpatient program options, thanks to Kimberly Butorac for the dotphrase

.JDBIRONDEFICIENCY - Brief summary of iron deficiency diagnostic criteria

.JDBLFTHIGH - Tiered approach to workup for elevated LFTs

.JDBLIPEDEMA - Brief tips on diagnosis and treatment

.JDBMELANOMANAIL - For high risk features of subungual melanoma

.JDBMIGRAINEPPX - brief migraine prophylaxis tips from the Headache Clinic

.JDBOSTEO - Brief new osteoporosis dx HPI/workup vanishing tip

.JDBPAIN - OTC pain treatments

.JDBPENEURODETAILS - Detailed neuro physical exam template, with vanishing tip (all my physical exam dotphrases start with .JDBPE)

.JDBPFT - Brief interpretation vanishing tip

.JDBPORPHYRIA - Brief tips on diagnosis and treatment

.JDBPREVENT - PREVENT cardiovascular risk calculator

.JDBRHINITIS - Chronic rhinitis A&P

.JDBSCURVY - Brief tips on diagnosis and treatment

.JDBSMARTLINKS - List of system smartlinks

.JDBTINNITUS - Brief HPI vanishing tip

Astrid Strasik:

.ASHPILIST - Smartlist of HPI dotphrases for common outpatient chief concerns in fill in format

To use the smartlist:

Add all dotphrases that start with .ASHPI

(click first one and hold shift while clicking last one to select and add all at once)

Then type .ASHPILIST.

AVS dotphrases: start with .ASAVS

.ASAVSHTNNEW - HTN AVS

.HEADACHERECS - Migraine treatment

James Vary:

Dermatology AVS dotphrases: start with .AVS

Epic Shortcuts:

= in orders fields enters **last attending physician** typed

Control + L - Opens Length of Service

Control + Shift + E - Sign Visit

Control + 1, 2, 3, etc. - Opens top tabs

Control + shift + 1, 2, 3, etc. - Switches windows

Any underlined letter is a keyboard shortcut -> **Alt + letter**

Right click - To accept order (anywhere in window)

Alt + s - Accept and sign order (bypasses accept)

Shift + F2 - Goes to last *** (opposite of F2)

Shift + tab - Goes to last tab section (opposite of tab)

t+30 in orders fields - Enters **date 30 days from today**, can type any number

QuickAction has "Do not allow replies" option. Recommend that you add this as default.

Schedule > Notes - Add notes for others to see on the schedule, such as instructions for Medical Assistants

Schedule > Events > Send Provider Late - Notify patient if they are signed in to telemedicine visit

You can move the Notes button to the left by clicking and dragging

Add patient profile picture - in Haiku, tap on the patient initials circle, same way as adding Media images



Add photo to note - click the icon on the right side of the top toolbar in the Note section (highlighted below)



Control + f - Search chats



vaccines today, ready in a couple minutes.
thanks!



Rule of 📖: If it's a basic keyboard shortcut on a PC (such as ctrl + f), it probably works in Epic too

Global search > trend - Trend lab values and vitals





Synonyms - Create shortcuts to dotphrases

- Open dotphrase > Synonyms dropdown on right > type in shortcut
- Override system dotphrase if needed
- Example: dotphrase .JDBBELLAB; synonym .L
 - Only type .l (lower case L) to pull in dotphrase (lab location dotphrase)

User SmartPhrase – JDBBELLAB [987142]

Synonyms	
	Synonym
1	L
2	

Order Sets can be created for the outpatient setting. Before signing order: **Options > Create Panel**

- These can be used to save settings for **referrals** and **procedures** as well.

Dexamethasone suppression test, low dose order set - Created by Dr. Karl Reis

How to add access:

Epic > Preference List Composer > Copy User Lists (at bottom) > enter user's name (my name)
This method gets around the limitation of only being able to follow 3 people on Epic.

Alternative:

Open patient chart > Order Search (Menu icon to right of Add Order) > Follow Users > Enter user's name

Dragon Shortcuts:

F2 - Jumps to next ***

New line - Jumps to new line

New paragraph - Jumps to new paragraph

Open parenth - Adds (

Closed parenth - Adds)

CPRS High Yield Shortcut:

Alt + f + i - Quickly sign all orders. Be sure to have *?* in your note template or will sign note too!

Patient-Centered Language Tips

[Open Notes tip of the week document](#)

A guide to using patient-centered language for notes.

By Christopher Wong, MD. Full text: Jackson S, Wong CJ, Editors. The Patient-Centered Approach to Medical Note-Writing (2024). New York: Springer.

[“In the Room” Language for Keeping Time.docx](#)

Brief guide on language for use with patients to help stay on time in clinic.

By Gena Lenti, MD.